From CBT to Christian CBT: A Brief History

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Cognitive-behavioral therapy (CBT) grew from two mid-century streams, behavior therapy and the cognitive therapies of Albert Ellis and Aaron Beck, into the most-studied psychotherapy in the world. Christian clinicians began adapting it in the late 1970s, and Siang-Yang Tan's 2007 article on prayer and Scripture gave that integration a clear, ethical method. This overview traces the line from the founders to today's research on Christian cognitive-behavioral therapy (CCBT), the tradition in which Belief Counseling stands.

Roots: behavior therapy and the cognitive turn

The behavioral half of CBT came from learning research. Pavlov's classical conditioning, Watson's behaviorism, and Skinner's operant conditioning showed that emotional and behavioral patterns are learned and can be relearned. In the 1950s Joseph Wolpe turned these ideas into systematic desensitization for fears, and behavior therapy became a treatment with measurable, testable procedures.

By the 1960s many clinicians found strict behaviorism too narrow, because it set aside what people think. Albert Bandura's social learning theory brought expectations and self-efficacy back into the picture. The wider "cognitive revolution" in psychology made it respectable to study thoughts directly. The ancient Stoic insight, often quoted by both Ellis and Beck from Epictetus, captured the new emphasis: people are disturbed less by events than by the views they take of them.

The founders and their core ideas

Albert Ellis began practicing "rational therapy" in the mid-1950s after growing frustrated with psychoanalysis. *Reason and Emotion in Psychotherapy* (Ellis, 1962) set out his ABC model: an **A**ctivating event does not by itself cause the emotional **C**onsequence; the **B**elief about the event does. Ellis taught clients to **D**ispute irrational beliefs, especially rigid demands ("I must," "they should") and catastrophizing, and replace them with rational ones. His approach became rational emotive behavior therapy (REBT).

Aaron T. Beck, a psychiatrist trained in psychoanalysis, set out to test the psychoanalytic view of depression and found instead that depressed patients showed a consistent negative bias in their thinking (Beck, 1963, 1964). He described:

- **Automatic thoughts**: quick, believable thoughts that pop up in response to events.
- **Cognitive distortions**: systematic errors such as overgeneralization, all-or-nothing thinking, and personalization.
- **The cognitive triad**: a negative view of the self, the world, and the future.
- **Schemas** or core beliefs: deep, enduring assumptions that shape what a person notices and how they interpret it.

Cognitive Therapy of Depression (Beck et al., 1979) turned these ideas into a structured, time-limited, collaborative treatment. Therapist and client treat beliefs as hypotheses, test them against evidence, and run behavioral experiments. Beck called this "collaborative empiricism."

Donald Meichenbaum joined the cognitive and behavioral strands. His self-instructional training and stress inoculation taught clients to change what they say to themselves while practicing new behavior (Meichenbaum, 1977). By the late 1970s these strands had merged under the name cognitive-behavioral therapy.

How CBT grew

From the 1980s on, CBT spread from depression to nearly every common disorder. Researchers built disorder-specific models, such as David Clark's account of panic as the catastrophic misreading of body sensations (Clark, 1986), and tested each protocol in randomized trials. A review of 269 meta-analyses found strong support for CBT across anxiety disorders, somatoform disorders, bulimia, anger, and general stress, with more mixed results for some other problems (Hofmann et al., 2012). Looking back over 40 years, Beck (2005) noted that the cognitive model had been supported for a wide range of conditions.

Two later developments widened the approach:

- **Schema-focused work.** For long-standing personality problems, Beck's students and colleagues such as Jeffrey Young developed schema therapy, which targets core beliefs formed in childhood.
- **The "third wave."** Acceptance and commitment therapy, dialectical behavior therapy, and mindfulness-based cognitive therapy added acceptance, mindfulness, values, and a changed relationship to thoughts, rather than only disputing their content (Hayes, 2004).

Two features of this history matter for Christian counselors. CBT has always insisted that beliefs matter and can be examined against evidence. It also grew by testing each adaptation, which set the standard later applied to faith-integrated versions.

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Early Christian integration (1980–2000)

Christian clinicians saw a natural fit. Scripture already speaks of renewing the mind (Romans 12:2), taking thoughts captive (2 Corinthians 10:5), and the truth that sets people free (John 8:32). CBT gave them a structured way to help clients examine beliefs and replace lies with truth.

- **Rebecca Propst** ran the first experiments. She tested religious versus nonreligious imagery for mildly depressed religious clients (Propst, 1980). She then compared religious CBT, standard CBT, pastoral counseling, and a waitlist for clinical depression. Religious CBT and pastoral counseling did better than standard CBT and the waitlist after treatment, though the groups were small and the differences faded by follow-up (Propst et al., 1992).
- **William Backus** popularized the approach for lay readers. *Telling Yourself the Truth* (Backus & Chapian, 1980) taught Christians to identify "misbeliefs" and replace them with biblical truth.
- **Mark McMinn** showed counselors how to adapt cognitive techniques within a Christian framework (McMinn, 1991).
- **Siang-Yang Tan** offered a biblical critique of secular CBT. He reviewed its limitations from both psychological and biblical viewpoints and proposed a biblical approach that is mainly cognitive-behavioral but broader and more comprehensive than secular CBT (Tan, 1987).

An inpatient study at the end of this period compared Christian and secular CBT programs for depressed Christian adults. Both groups improved in depression and spiritual well-being, and spiritual well-being scores were higher overall in the Christian program (Hawkins et al., 1999).

Tan's seminal article: prayer and Scripture in CBT (2007)

Tan's "Use of Prayer and Scripture in Cognitive-Behavioral Therapy" (Tan, 2007) gave Christian CBT a practical, ethical method. It placed CCBT alongside the newer third-wave therapies and answered the question many counselors were asking: how, exactly, do prayer and Scripture fit into a session?

Implicit and explicit integration. Tan described a continuum. In *implicit* integration, the counselor's faith shapes the work but spiritual topics are not raised or used openly. In *explicit* integration, the counselor directly addresses spiritual issues and uses resources such as prayer and Scripture. Either can be appropriate if the counselor integrates intentionally. Explicit work requires the client's informed consent, gathered at intake along with the client's religious background.

Features of a biblical CBT. Tan listed eight, including a warm therapeutic relationship, attention to past experiences, openness to the spiritual and experiential, reliance on the Holy Spirit, attention to family, church, and culture, use only of techniques consistent with biblical truth, and rigorous outcome research.

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Scripture in cognitive restructuring. Standard CBT questions ("What is the evidence for this thought?") are joined by faith-based ones ("What does Scripture say about this belief?"). Tan stressed reading passages in context and for their purpose, so that Scripture corrects distorted beliefs rather than being used as a club. His case example showed Scripture helping a client separate healthy anger from destructive bitterness.

Prayer, including inner healing prayer. Tan outlined a seven-step inner healing prayer for painful memories: opening prayer, relaxation, recalling the memory, inviting Christ's ministry, waiting quietly, closing prayer, and debriefing. He cautioned that it is not a cure-all and that healing may take more than one session.

Ethical cautions. Prayer and Scripture may not suit clients who are severely disturbed or psychotic until they are stable, clients too angry at God to engage, or clients who decline. Some settings, such as public clinics and schools, limit explicit spiritual work, and minors need appropriate consent. Clients who do not want explicit integration receive standard CBT.

Research base. Tan counted at least ten outcome studies of religiously oriented CBT, six Christian and four Muslim, with support strongest for depression and weaker for generalized anxiety. He called for research on inner healing prayer itself.

The research since Tan

After 2007 the question shifted from "Can faith be integrated?" to "How well does integrated therapy work, and for whom?"

- **Systematic reviews.** A review of religious and spiritual therapies found several helpful for clients but limited evidence that they outperformed established secular therapies, so the choice "may be an issue of client preference and therapist comfort" (Hook et al., 2010). A meta-analysis of 46 studies (3,290 participants) found religiously accommodated therapies did better than secular alternatives on psychological ($d = 0.26$) and spiritual ($d = 0.41$) outcomes. In the strictest comparisons, with the same therapy with or without religious content, they did better on spiritual but not psychological outcomes (Worthington et al., 2011).
- **Applying Scripture.** Shortly before Tan's article, Fernando Garzon described specific ways counselors use Scripture in therapy and called for research on them (Garzon, 2005).
- **Religiously integrated CBT (RCBT).** Michelle Pearce, Harold Koenig, and colleagues at Duke wrote a manualized RCBT for major depression in people with chronic medical illness, with versions for several faith traditions, including Christianity (Pearce et al., 2015). In a randomized trial of 132 adults, RCBT and conventional CBT produced equivalent results overall, and RCBT was somewhat more effective for more religious participants (Koenig et al., 2015).

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- **Faith-adapted CBT for depression and anxiety.** A meta-analysis of 16 trials found significant benefits but small samples and a high risk of bias, so firm recommendations await larger trials (Anderson et al., 2015).
- **The broadest meta-analysis.** Across 97 studies, therapy adapted to clients' religion or spirituality beat no treatment and modestly beat secular therapy. In the most rigorous designs it matched secular therapy on psychological distress and did better on spiritual well-being (Captari et al., 2018).

The consistent picture: Christian CBT works about as well as standard CBT for reducing distress, adds spiritual benefits, and is often preferred by religious clients. The evidence is real but still modest.

Christian CBT today

Contemporary Christian CBT has three marks.

1. **It is broader than disputing thoughts.** Following the third wave, Christian clinicians now draw on contemplative practices. In a multisite trial, a four-week Christian meditation program reduced repetitive negative thinking among Christian university students compared with a waitlist (Knabb et al., 2020). Tan's textbook, now in its second edition, surveys the major therapies, CBT among them, from a Christian perspective (Tan, 2022).
2. **It is client-centered and ethical.** Integration follows the client's wishes, a spiritual assessment, and informed consent, as Tan (2007) urged. Explicit use of prayer and Scripture is offered, never imposed.
3. **It is accountable to evidence.** Christian approaches are tested the same way as secular ones, and honest reports note where the evidence is still thin.

Where Belief Counseling fits. Belief Counseling stands squarely in this line. From Ellis and Beck it takes the conviction that beliefs drive emotion and behavior and can be examined. From Tan and the CCBT tradition it takes the explicit, consent-based use of Scripture and prayer. Its three steps follow that lineage: **Identify** the false belief (including core beliefs and parental messages), **Challenge** it with questions, evidence, and behavioral experiments, and **Replace** it with the truth of God's Word.

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